

PRIOR AUTHORIZATION REQUEST

Cover Letter

Patient Name	John Smith
Date of Birth	1968-03-15
Insurance	United Healthcare
Member ID	UHC-887612
Policy Number	UHC-POL-2026-001
CPT Code	75563
ICD-10	I42.0
Physician	Dr. Ramesh Kumar
Physician NPI	1234567890
Facility	City Heart Institute
Date of Request	March 18, 2026

March 18, 2026

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

****RE: Prior Authorization Request for Cardiac MRI with Contrast (CPT 75563)****

****Patient Name:** John Smith**

****Date of Birth:** 1968-03-15**

****Insurance Member ID:** UHC-887612**

****Requesting Physician:** Dr. Ramesh Kumar, NPI: 1234567890**

****Specialty:** Cardiology**

****Facility:** City Heart Institute**

Dear Prior Authorization Review Team,

This letter is to request prior authorization for a Cardiac MRI with Contrast (CPT 75563) for our patient, John Smith. Mr. Smith is a 58-year-old male with a diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0) and has been under our care for approximately three years. Despite adherence to Guideline-Directed Medical Therapy (GDMT), including Carvedilol, Lisinopril, and Spironolactone, Mr. Smith continues to experience significant symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Recent diagnostic evaluations, including an echocardiogram performed in January 2026, revealed a severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% and diffuse global hypokinesia. Furthermore, laboratory results

from March 10, 2026, indicate a significantly elevated B-type Natriuretic Peptide (BNP) level of 450 pg/mL, further supporting the presence of advanced heart failure. The requested Cardiac MRI with Contrast is medically necessary to provide definitive tissue characterization, assess myocardial fibrosis via Late Gadolinium Enhancement (LGE), differentiate between ischemic and non-ischemic cardiomyopathy, and evaluate myocardial viability. These critical findings will directly inform future treatment decisions, including the potential need for revascularization or implantable cardioverter-defibrillator (ICD) placement, and are essential for optimizing Mr. Smith's prognosis and management.

Sincerely,

Dr. Ramesh Kumar MD

Cardiology

NPI: 1234567890

City Heart Institute

Clinical Summary

Clinical Summary for Prior Authorization Request

1. Diagnosis:

* **ICD-10 Code:** I42.0

* **Full Description:** Dilated Cardiomyopathy

2. Patient History:

Mr. John Smith is a 58-year-old male with a 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). He has a history of worsening shortness of breath on exertion, fatigue, and reduced exercise tolerance, consistent with New York Heart Association (NYHA) Class III heart failure. He also reports occasional palpitations. Comorbidities include heart failure with reduced ejection fraction (ICD-10: I50.20).

3. Reason for Procedure:

* **CPT Code:** 75563

* **Description:** Cardiac MRI with contrast

This procedure is requested to obtain definitive tissue characterization of the myocardium, assess for myocardial fibrosis via Late Gadolinium Enhancement (LGE), differentiate between ischemic and non-ischemic cardiomyopathy, and evaluate myocardial viability. These findings are critical for guiding future high-stakes treatment decisions, including potential revascularization candidacy and consideration for ICD implantation.

4. Supporting Evidence:

* **Echocardiogram (January 15, 2026):** Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis. Moderate LV dilation. Grade II diastolic dysfunction.

* **BNP (March 10, 2026):** 450 pg/mL (elevated, reference < 100 pg/mL)

* **NT-proBNP (March 10, 2026):** 1850 pg/mL (elevated, reference < 125 pg/mL for age < 75)

* **Symptoms:** NYHA Class III, shortness of breath at rest and with minimal exertion, orthopnea requiring 3-pillow support, bilateral lower extremity edema.

* **Prior Treatments Failed:** Despite adherence to Guideline-Directed Medical Therapy (GDMT) including Carvedilol (25mg BID), Lisinopril (10mg daily), and Spironolactone (25mg daily) for over three years, the patient's symptoms persist and have shown recent signs of progression. Sacubitril/Valsartan and Ivabradine were discontinued due to side effects.

5. Clinical Justification:

Cardiac MRI with contrast is medically necessary for Mr. Smith due to the clinical insufficiency of echocardiography in providing detailed myocardial tissue characterization. Standard echocardiography is unable to fully assess myocardial viability or definitively rule out infiltrative cardiomyopathies. The requested Cardiac MRI with contrast is essential for obtaining high-resolution data, including LGE imaging, which is crucial for accurate diagnosis, prognosis, and guiding optimal therapeutic strategies in patients with dilated cardiomyopathy and persistent symptoms despite optimal medical management. This aligns with current cardiology guidelines for the comprehensive evaluation of cardiomyopathy.

Requirements Checklist

Requirement	Status	Evidence
Confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0)	✓ MET	Primary ICD-10 code is I42.0, and the diagnosis is listed as Dilated Cardiomyopathy in the EHR. The Clinical Justification Note also confirms this diagnosis.
Symptoms present and documented for a minimum of 3 months	✓ MET	The Clinical Justification Note states a '3-year history of Dilated Cardiomyopathy' and describes 'worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance'. The Prior Treatment Records also cover a period from Jan 2023 to Mar 2026, indicating a long-standing condition.
Shortness of breath on exertion documented	✓ MET	The Clinical Justification Note states 'worsening shortness of breath on exertion' and 'shortness of breath both at rest and upon minimal physical exertion'.
Chest pain or pressure documented	✗ MISSING	Chest pain or pressure is not explicitly mentioned in the provided EHR data. The Clinical Justification Note mentions 'occasional palpitations'.
Palpitations or arrhythmia documented	✓ MET	The Clinical Justification Note states 'the patient reports occasional palpitations'.
Unexplained syncope or near-syncope documented	✗ MISSING	Syncope or near-syncope is not mentioned in the provided EHR data.
Reduced exercise tolerance documented	✓ MET	The Clinical Justification Note states 'a marked reduction in exercise tolerance that interferes with activities of daily living'.
Echocardiogram completed prior to MRI request	✓ MET	An Echocardiogram Report dated January 15, 2026, is provided, and the MRI request is dated March 16, 2026.
Echocardiogram shows LVEF documented value	✓ MET	The Echocardiogram Report clearly states 'LV Ejection Fraction (LVEF) 35%'.
Echocardiogram shows evidence of wall motion abnormality	✓ MET	The Echocardiogram Report states 'Diffuse global hypokinesis noted'.
Echocardiogram shows structural cardiac abnormality requiring further characterization	✓ MET	The Echocardiogram Report concludes with '2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.' and 'Recommend Cardiac MRI for further tissue characterization.'
Request ordered by a board-certified Cardiologist or Cardiac Electrophysiologist	✓ MET	Dr. Ramesh Kumar is listed as the physician_name with specialty 'Cardiology' and is described as 'Board Certified Cardiologist' in multiple documents.

Clinical Justification Note from treating Cardiologist explaining medical necessity	✓ MET	A 'Clinical Justification Note' from Dr. Ramesh Kumar MD, dated March 16, 2026, is uploaded and details patient history, current symptoms, previous treatments, and the insufficiency of echocardiography.
Echocardiogram Report within last 12 months	✓ MET	The Echocardiogram Report is dated January 15, 2026, which is within 12 months of the request date (March 16, 2026).
Echocardiogram Report documents LVEF value	✓ MET	The Echocardiogram Report documents 'LV Ejection Fraction (LVEF) 35%'.
Echocardiogram Report documents chamber dimensions	✓ MET	The Echocardiogram Report includes 'LV End Diastolic Diameter 6.2 cm' and 'LV End Systolic Diameter 5.1 cm'.
Echocardiogram Report documents wall motion analysis	✓ MET	The Echocardiogram Report states 'Wall Motion: Diffuse global hypokinesis noted.'
Echocardiogram Report documents valve function	✓ MET	The Echocardiogram Report details 'Mitral Valve: Mild mitral regurgitation identified.' and 'Tricuspid Valve: Mild tricuspid regurgitation.'
Recent Lab Results within last 30 days	✓ MET	The 'Recent Lab Results' report is dated March 10, 2026, which is within 30 days of the request date (March 16, 2026).
BNP or NT-proBNP levels documented	✓ MET	The 'Recent Lab Results' report includes 'BNP (B-type Natriuretic Peptide) 450 pg/mL' and 'NT-proBNP 1850 pg/mL'.
Complete metabolic panel documented	✓ MET	The 'Recent Lab Results' report includes a 'COMPREHENSIVE METABOLIC PANEL' with results for Sodium, Potassium, Creatinine, eGFR, BUN, Glucose, AST, ALT, and Albumin.
Complete blood count documented	✓ MET	The 'Recent Lab Results' report includes a 'COMPLETE BLOOD COUNT' with results for WBC, RBC, Hemoglobin, Hematocrit, Platelets, and MCV.
Physician Order Letter on physician letterhead signed by referring Cardiologist	✓ MET	A 'Physician Order Letter' from Dr. Ramesh Kumar MD, dated March 16, 2026, is uploaded and includes his letterhead, signature, and NPI.
Physician Order Letter includes procedure requested	✓ MET	The 'Physician Order Letter' clearly states 'PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)'.
Physician Order Letter includes ICD-10 diagnosis code	✓ MET	The 'Physician Order Letter' lists 'ICD-10 CODE: I42.0'.
Physician Order Letter includes clinical indication	✓ MET	The 'Physician Order Letter' lists 'Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)' and 'MEDICAL NECESSITY STATEMENT' detailing the reasons for the procedure.

Physician Order Letter includes physician NPI number	✓ MET	The 'Physician Order Letter' includes 'NPI: 1234567890'.
Prior Treatment Records documentation of cardiac medications tried for minimum 3 months	✓ MET	The 'Prior Treatment Records' document covers 'Jan 2023 – Mar 2026', which is a 3-year period. It lists medications like Carvedilol, Lisinopril, and Spironolactone initiated in Jan 2023 or Mar 2023.
Prior Treatment Records include drug names	✓ MET	The 'Prior Treatment Records' list 'Carvedilol', 'Lisinopril', 'Spironolactone', 'Furosemide', 'Sacubitril/Valsartan', and 'Ivabradine'.
Prior Treatment Records include dosages	✓ MET	The 'Prior Treatment Records' list dosages such as '25mg', '10mg', '25mg', '40mg', '24/26mg', and '5mg'.
Prior Treatment Records include duration of therapy	✓ MET	The 'Prior Treatment Records' specify durations like '3 years', '2y 9mo', and '2 months'.
Prior Treatment Records include patient response or reason for discontinuation	✓ MET	The 'Prior Treatment Records' detail responses like 'Partial response — HR controlled but symptoms persist', 'Partial response — BP controlled but LVEF unchanged', and reasons for discontinuation such as 'Discontinued due to symptomatic hypotension and dizziness'.

Attached Documents

The following documents are attached to this package:

#		
1	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
2	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
3	Laboratory Results Report	Laboratory_Results_Report.pdf
4	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
5	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf

Physician: Dr. Ramesh Kumar MD

NPI Number: 1234567890

Date: March 16, 2026

RE: PRIOR AUTHORIZATION REQUEST FOR CARDIAC MRI WITH CONTRAST (CPT 75563)

PATIENT HISTORY

The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Initially diagnosed in January 2023 following a presentation with acute dyspnea, the patient has since exhibited a chronic and progressive clinical course. Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance that interferes with activities of daily living. Furthermore, the patient reports occasional palpitations, necessitating advanced diagnostic evaluation to assess for structural triggers or arrhythmogenic substrates.

CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

PREVIOUS TREATMENTS TRIED

The patient has remained strictly compliant with Guideline-Directed Medical Therapy (GDMT) since diagnosis. The therapeutic regimen includes the following:

- Carvedilol: 25mg twice daily (initiated January 2023)
- Lisinopril: 10mg daily (initiated January 2023)
- Spironolactone: 25mg daily (initiated March 2023)
- Furosemide: 40mg daily for fluid management

Despite adherence to optimal medical therapy for over three years, the patient's symptoms persist and have shown recent signs of progression, suggesting the underlying pathology requires more granular characterization than what is provided by current management tools.

CLINICAL INSUFFICIENCY OF ECHOCARDIOGRAPHY

A transthoracic echocardiogram (TTE) performed in January 2026 documented a Left Ventricular Ejection Fraction (LVEF) of 35% with global hypokinesis. While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for

definitive tissue characterization and to provide the high-resolution data necessary to guide future high-stakes treatment decisions.

MEDICAL NECESSITY STATEMENT

Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy and to differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging will directly impact the clinical treatment plan, influence prognosis, and determine the appropriateness of further interventional therapies.

Respectfully,

Dr. Ramesh Kumar MD
Date Signed: March 16, 2026
Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

TRANSTHORACIC ECHOCARDIOGRAM REPORT

Ordering Physician:

Dr. Ramesh Kumar MD

Performing Physician:

Dr. Sarah Johnson MD

Facility:

City Heart Institute

Clinical Indication:

Dilated Cardiomyopathy; Follow up evaluation

Study Date:

January 15, 2026

Report Date:

January 15, 2026

Study Type:

Complete TTE

SECTION 1: TECHNICAL QUALITY

Study Quality: Adequate

Windows: Acceptable acoustic windows obtained.

Views: Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

SECTION 2: LEFT VENTRICLE MEASUREMENTS

Measurement	Result	Normal Reference Range
LV End Diastolic Diameter	6.2 cm	Less than 5.5
LV End Systolic Diameter	5.1 cm	Less than 3.9
Interventricular Septum	0.8 cm	0.6 to 1.1
Posterior Wall Thickness	0.8 cm	0.6 to 1.1
LV Ejection Fraction (LVEF)	35%	Greater than 55%
LV Fractional Shortening	18%	25 to 45%

SECTION 3: LEFT VENTRICLE FUNCTION

LVEF: 35% – Severely reduced.

Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.

Diastolic Function: Grade II diastolic dysfunction (pseudonormal pattern).

LV Size: Moderately dilated left ventricle.

SECTION 4: RIGHT VENTRICLE

RV Size: Normal.

RV Function: Mildly reduced.

TAPSE: 1.6 cm (mildly reduced).

RV Systolic Pressure: Estimated 38 mmHg.

SECTION 5: VALVES

Mitral Valve: Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.

Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.

Tricuspid Valve: Mild tricuspid regurgitation.

Pulmonic Valve: Normal appearance and function.

SECTION 6: OTHER FINDINGS

Pericardium: Small pericardial effusion noted.

Aorta: Normal aortic root diameter.

IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. Severely reduced left ventricular systolic function with LVEF of 35%.
2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.
3. Diffuse global hypokinesis.
4. Mild mitral regurgitation — functional etiology.
5. Grade II diastolic dysfunction.
6. Small pericardial effusion.
7. Recommend Cardiac MRI for further tissue characterization.

Interpreting Physician: Dr. Sarah Johnson MD

Board Certified Echocardiographer

Date: January 15, 2026

Ordering Physician:	Dr. Ramesh Kumar MD	Specimen Type:	Venous Blood
Collection Date:	March 10, 2026	Clinical Indication:	Dilated Cardiomyopathy
Report Date:	March 10, 2026		Cardiac MRI Pre-authorization

SECTION 1 - CARDIAC BIOMARKERS

Test	Result	Unit	Reference Range	Flag
BNP (B-type Natriuretic Peptide)	450	pg/mL	Less than 100	HIGH
NT-proBNP	1850	pg/mL	Less than 125 (age < 75)	HIGH
Troponin I (High Sensitivity)	0.04	ng/mL	Less than 0.04	BORDERLINE

SECTION 2 - COMPLETE BLOOD COUNT

Test	Result	Unit	Reference Range	Flag
WBC	7.2	10^3/uL	4.5 to 11.0	NORMAL
RBC	4.8	10^6/uL	4.5 to 5.9	NORMAL
Hemoglobin	13.8	g/dL	13.5 to 17.5	NORMAL
Hematocrit	41.2	%	41 to 53	NORMAL
Platelets	215	10^3/uL	150 to 400	NORMAL
MCV	88	fL	80 to 100	NORMAL

SECTION 3 - COMPREHENSIVE METABOLIC PANEL

Test	Result	Unit	Reference Range	Flag
Sodium	138	mEq/L	136 to 145	NORMAL
Potassium	4.1	mEq/L	3.5 to 5.1	NORMAL
Creatinine	1.1	mg/dL	0.7 to 1.3	NORMAL
eGFR	72	mL/min	Greater than 60	NORMAL
BUN	18	mg/dL	7 to 20	NORMAL

Test	Result	Unit	Reference Range	Flag
Glucose	95	mg/dL	70 to 100	NORMAL
AST	28	U/L	10 to 40	NORMAL
ALT	22	U/L	7 to 56	NORMAL
Albumin	3.8	g/dL	3.5 to 5.0	NORMAL

SECTION 4 - LIPID PANEL

Test	Result	Unit	Reference Range	Flag
Total Cholesterol	195	mg/dL	Less than 200	NORMAL
LDL	118	mg/dL	Less than 130	NORMAL
HDL	42	mg/dL	Greater than 40	NORMAL
Triglycerides	175	mg/dL	Less than 150	BORDERLINE HIGH

SECTION 5 - INTERPRETATION NOTE

Significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL consistent with moderate to severe heart failure.
Renal function preserved. Electrolytes within normal limits.
Results support ongoing heart failure management and prior authorization for advanced cardiac imaging.

Authorized By:

Dr. Michael Chen MD
Board Certified Pathologist
Date: March 10, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001

Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

From:

Dr. Ramesh Kumar MD

Board Certified Cardiologist

NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST

PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)

ICD-10 CODE: I42.0

REQUESTED PROCEDURE

Name:	Cardiac MRI with Contrast
CPT Code:	75563
Urgency:	Routine — Non Emergency
Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)

Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

Treating Physician:	Dr. Ramesh Kumar MD	Report Date:	March 16, 2026
Facility:	City Heart Institute	Specialty:	Cardiology
Period Covered:	Jan 2023 – Mar 2026	Clinical Indication:	Dilated Cardiomyopathy

This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.

SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response – HR controlled but symptoms persist	CURRENT
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response – BP controlled but LVEF unchanged	CURRENT
Spirololactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	CURRENT
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	CURRENT
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	DISCONTINUED
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued – inadequate heart rate reduction	DISCONTINUED

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:

I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD

Board Certified Cardiologist | NPI: 1234567890

Date: March 16, 2026